First 90 days: a pilot proposal

Dr Mark Aszkenasy, for discussion with Care ADHD. Monday 12 October 2026. The concept demo uses synthetic data only.

Why now. The final Fonagy report, Independent review into mental health conditions, ADHD and autism, was published on GOV.UK on 9 October 2026. Independent providers carry out over half of NHS-funded ADHD assessments, and commissioners often cannot see what they are paying for. The review recommends making all ADHD and autism diagnostic assessments CQC-regulated, with stronger national oversight (Rec 5). The review asks for timely medication reviews, safe deprescribing, long-term management in primary care where required, and outcomes measured by participation and quality of life, not activity. This pilot produces that assurance evidence for commissioners and CQC. **We can show commissioners exactly what they're paying for.**

Three objectives

For confirmation: the routine data work serves all three of Care ADHD's organisational objectives. **Safety:** safer, better-quality care, with NICE NG87 monitoring done reliably for every child. **Experience:** a better experience of care for families, with clearer letters, fewer missed checks and shorter waits. **Growth:** more children seen with the same clinical time, because the audit covers every record each night and scales without adding clinic time.

The proposal

Take the nightly NICE audit and safety worklist from the demo into live use on one pathway: **children and young people on ADHD medication, from titration through stable dose to shared care with the GP**. Prove it there, then extend. Why this first: it turns retrospective audit into continuous assurance, with clinicians making every decision. The rules come from NG87 and are explainable, with no machine learning. It runs on data we already hold, and a rule-based tool with clinician sign-off is a straightforward DPIA and clinical safety case.

What the audit checks

- **Titration and follow-up:** the NG87 monitoring checks (height, weight, heart rate, blood pressure, response and side effects) recorded when due.
- **Stable dose** (same rule engine and worklist, added in weeks 3 to 10):
  - **Annual review:** flagged at 11 months, before it falls overdue (NG87 1.10.1).
  - **Treatment break:** a prompt to record that a break has been considered and discussed with the family. A prompt, not a push.
  - **Shared-care readiness:** stable dose for at least 3 months; monitoring complete and in range; no unresolved side effects; GP agreement; family understanding. This gives a 'ready to transfer' list, and the specific gaps for children who are nearly ready.
  - **After transfer:** a check that GP monitoring results come back to us.

Value agreed up front (Principle 2)

We fix the measures and the baseline in weeks 0 to 2, before anything goes live. Targets are agreed then, not guessed now.

- **Monitoring completeness:** proportion of children with the NG87 checks recorded when due. Baseline vs day 90.
- **Annual reviews on time:** % of children on a stable dose reviewed within 12 months.
- **Shared care:** % of eligible children transferred to shared care.
- **Time to action:** days from a standard being missed to a clinician acting on it.
- **Clinician time and flag accuracy:** minutes per flag, and the share of flags clinicians judge to be a true miss.
- **Safety:** incidents or near misses linked to the tool, from the hazard log.
- **National data completeness:** % of children in the pilot with a coded outcome score, medication review and diagnosis date in our MHSDS return. Baseline vs day 90.
- **One outcome measure:** a participation or quality-of-life measure baselined in weeks 0 to 2 and repeated at review, such as school attendance or a functional score like WFIRS-P. Ninety days sets the baseline; change takes longer.

The whole journey (Principle 1)

- **Overnight** the audit runs on pseudonymised data. By the UK morning the **treating clinician** sees their own patients' flags with the standard, the evidence and the gap, and signs off each one: action taken, or not applicable with a reason.
- **Next night:** the audit confirms whether the gap has closed. Unactioned flags escalate to the pathway clinical lead after an agreed interval.
- **GP** receives a shared-care request only for children on the ready list. Returned results are checked nightly.
- **Pathway lead** sees compliance by standard and team. **Clinical governance committee** gets a monthly report. **Board and commissioners** see a short assurance summary.

Governance from day one

- **DPIA** started in week 0 and signed off before live use, covering the UK to India data flow. **Caldicott Guardian** and **DPO** sign off data access. Identifiable data stays in the UK unless the DPIA approves otherwise.
- **DCB0129 and DCB0160** both apply, as we build and deploy the tool ourselves. Safety case and hazard log opened in week 1 and kept live. A named **Clinical Safety Officer**, ideally not the rule author, so review is independent.
- **MHRA classification:** a documented decision on whether the tool is software as a medical device. We confirm this rather than assume it.

National data returns

- **One data model, three uses.** Most of the fields the audit reads are ones NHS England already requires from us. If we build it once, it serves the worklist, the commissioner report and the national return.
- **MHSDS.** NHS-funded ADHD and autism work in a mental-health-commissioned service goes to the Mental Health Services Data Set. This is a Standard Contract requirement (SC28, Schedule 6A), and we already submit monthly. NHS England's technical guidance sets the coding: referral reason, suspected ADHD, ADHD or autism diagnostic assessment, diagnosis with the date given, medication review, and discharge. Where under-18s work is commissioned as community paediatrics, the Community Services Data Set (CSDS) applies instead.
- **Quick win.** Referrals, contacts and primary diagnoses already flow to MHSDS. Outcome scores, care plans and medication records are the next step. Without a baseline outcome, a child's national waiting-time clock does not stop, so recording these would make our waiting figures and our quality evidence more accurate.
- **Why it matters now.** The 2026/27 NHS Payment Scheme guidance tells ICBs to pay only when data standards are met. Fonagy (Rec 5) asks that all providers, including those under Right to Choose, submit consistent activity, quality and outcomes data as a condition of funding and accreditation. The review found only about 10 of some 120 ADHD providers submitting, and we are one of them.

Owners and champions (Principles 3, 4 and 5)

- **Executive sponsor:** CEO until the Director of AI and Data Science is in post.
- **Clinical lead for the pilot:** myself, as senior clinical adviser on an agreed sessional basis. I own the rule specifications and clinical sign-off of the logic. I have a long-standing interest in large data and clinical epidemiology. Before returning to paediatrics, I was a senior registrar in public health medicine at the Communicable Disease Surveillance Centre (CDSC), analysing the data that helped make the case for introducing the routine pneumococcal vaccine for children.
- **Product owner:** one named person who owns the NICE audit as a product after day 90, with a backlog and change control through the safety case.
- **Clinical champions:** three or four clinicians on the pathway, with protected time and funded clinical safety and data training. They shape the worklist and test it first.
- **Bengaluru GCC:** data engineering, rule build and testing, overnight run monitoring and data-quality checks, so the worklist is ready for the UK morning. This is follow-the-sun in practice. UK staff own clinical sign-off and act on flags.

Phased plan

Weeks	Work	Deliverables
0 to 2 Set up	Name owners. Fix scope and measures. Map data sources, including to MHSDS items, and check our current MHSDS completeness. Start DPIA. Hazard identification workshop. MHRA classification note. Measure the baseline, including the outcome measure.	Signed one-page value case. DPIA draft. Hazard log v0.1. Baseline figures. Audit-to-MHSDS data map.
3 to 6 Build and validate	Encode the titration monitoring rules, each signed off clinically. Run in shadow mode on historical data, with no flags shown. Compare against a manual audit sample. Specify and build the stable-dose rules. Design the worklist with champions.	Validated titration rules. Accuracy vs manual audit. Clinical safety case. DPIA approved. Go-live decision.
7 to 10 Live with champions	Titration rules live. Stable-dose rules through shadow mode, then live once validated. Champions sign off every flag. Weekly review of flags, false positives, time taken and hazards.	Weekly metrics. First 'ready to transfer' list. Updated hazard log. Training material.
11 to 13 Evaluate and decide	Compare against baseline, including MHSDS completeness. Clinician and GP feedback. Running cost. Safety review.	Evaluation and assurance report for the board and commissioners. Recommendation for day 90.

Decision at day 90

- **Scale:** extend across the children's ADHD pathway, then to adult ADHD, autism and the combined pathway.
- **Assessment quality audit.** Check each assessment for informant input from parent and school, developmental history, a face-to-face or observed element, screening for co-occurring conditions (autism, anxiety, sleep, learning difficulties) and review of prior records. The review links misdiagnosis risk to assessments that lack these. Much of this sits in letters, so letter extraction becomes the enabler rather than a separate project.
- **Needs-led waiting-list triage** (Rec 2). Urgency and complexity scoring from referral information. A clinician confirms every reprioritisation. Fairness checks across deprivation and ethnicity before go-live. This needs its own DPIA and a careful MHRA classification check.
- **Local language model.** Letter extraction and triage mean passing free-text clinical letters through a language model. We would run an open-weights model on Care ADHD's own UK infrastructure, so identifiable text never leaves our control or reaches an outside AI provider. Only extracted, pseudonymised fields would flow to Bengaluru. This simplifies the DPIA and the UK to India transfer question. It costs some accuracy against the large commercial models and needs in-house hosting, so we validate it against clinician-coded letters and record it in the safety case. A UK-hosted commercial model under a zero-retention contract is the fallback.
- **Adjust or stop:** if the measures do not move, or the burden on clinicians outweighs the benefit, we say so and change course.

Risks

- **Data quality.** Monitoring may sit in free-text letters, so the audit undercounts. Shadow mode measures this before go-live.
- **Alert fatigue.** Too many flags and clinicians stop reading them. We start narrow and track accuracy weekly. Every flag has a named owner and escalation route.
- **GP capacity.** Some practices will decline shared care. The readiness list shows where. It does not solve it.
- **Cross-border data.** India has no UK adequacy decision. Transfers are settled before identifiable data moves.
- **Key-person dependency.** I am part-time. The product owner and champions carry the day-to-day work.

For discussion on Monday

- My role, time commitment and reporting line, including once the Director is appointed.
- Who acts as Clinical Safety Officer and Caldicott Guardian for the pilot.
- Which clinical system and data we can access, and which commissioners should see the first assurance report.
- Who owns our MHSDS return, how complete it is, and whether any under-18s contract expects CSDS rather than MHSDS.